RIVERBEND SPINE & FAMILY MEDICINE
Office Visit Note

Patient: Marlene Kowalski DOB: 07/22/1968 MRN: 884129
Date of visit: 03/14/2026
Provider: Elena Voss, MD NPI: 1043729185

CHIEF COMPLAINT: Persistent low back pain with left leg radiation x 4 months.

HISTORY: 57-year-old female presents with worsening lumbar pain radiating to the left posterior thigh and calf, present since late November 2025. Pain rated 7/10, worse with sitting and forward flexion. She has completed eight (8) weeks of structured physical therapy (01/05/2026–02/27/2026) with minimal improvement. She has trialed NSAIDs (ibuprofen 600 mg TID x 6 weeks) and a course of cyclobenzaprine without durable relief. Pain now interferes with her ability to work full shifts as a warehouse supervisor.

EXAM: Antalgic gait. Lumbar flexion limited to 45 degrees with pain. Straight-leg raise positive on the left at 40 degrees, reproducing her calf pain. Left Achilles reflex diminished (1+) versus right (2+). Strength 4/5 left extensor hallucis longus. Sensation reduced over the left lateral foot. No saddle anesthesia, no bowel/bladder changes.

ASSESSMENT:
1. Low back pain with left-sided radiculopathy (M54.5)
2. Suspected L5-S1 disc herniation with early L5 radiculopathy (M51.26)

PLAN: Given eight weeks of failed conservative management and new progressive motor and reflex findings on exam, I am ordering an MRI of the lumbar spine without contrast (CPT 72148) to evaluate for disc herniation and nerve root compression. If imaging confirms a surgical lesion, will refer to neurosurgery. Continue home exercise program; return to work with 20-lb lifting restriction.

Elena Voss, MD
Electronically signed 03/14/2026